

Risk Summary

Preeclampsia

Medium Score: 0.6395082154771051

Model Interpretation: Risk-based triage only (not diagnosis).

Macrosomia

Medium Score: 0.5943816940585895

Model Interpretation: Risk-based coaching support.

Summary: Patient TEST_MED_MED, at 32 weeks gestation, presents with a medium risk for preeclampsia and medium risk for macrosomia. Key findings include an elevated systolic blood pressure of 142 mmHg and dipstick proteinuria of 1. Maternal age is 38 years and pre-pregnancy BMI is 42.4 kg/m^2. Fasting glucose is 99 mg/dL and 1-hour postprandial glucose is 126 mg/dL. An alert to the doctor is recommended due to the medium preeclampsia risk.

Patient Snapshot (Recent Inputs)

Parameter	Value	Notes
Blood Pressure	142/79 mmHg	Hypertensive threshold ≥140/90, severe ≥160/110.
Proteinuria (dipstick)	1	≥1+ supports PE suspicion in context of HTN.
Symptoms	Headache=0, Vision=0, RUQ Pain=0, Swelling=0	Severe features increase escalation priority.
Glucose	Fasting=99 mg/dL 1hr PP=126 mg/dL	Targets typically fasting <95, 1hr <140 (pregnancy goals).
HbA1c	5.6	Higher HbA1c indicates sustained glycemic elevation.
Maternal Context	Age=38 BMI=42.4 GDM=0	Risk modifiers for fetal overgrowth + metabolic risk.
Activity / Sleep	Post-meal walk=29 min Sleep=6.1 hrs	Lifestyle factors influence glycemic response.
Food Log	paneer salad	Used for dietary correlation patterns.

Reason for Alert

Action Requested: ALERT_DOCTOR

Clinical Rationale: ALERT: Patient TEST_MED_MED, 32 weeks gestation, has a MEDIUM preeclampsia risk tier (score 0.6395) and MEDIUM macrosomia risk tier (score 0.5944). Key vitals: Systolic BP 142 mmHg, Diastolic BP 79 mmHg, Proteinuria dipstick 1. Fasting glucose 99 mg/dL, 1-hr postprandial glucose 126 mg/dL. No severe symptoms (headache, vision changes, upper abdominal pain, facial/hand swelling) reported. Please review the patient's full record for further management.

Suggested Next Steps

- Review trend: BP, symptoms, proteinuria, glucose readings
- Consider confirmatory tests (protein/creatinine ratio, LFTs, platelets) if clinically indicated
- Consider GDM diet review or insulin adjustment if glucose targets exceeded
- Provide patient education + return precautions

